

DEPARTMENT OF NUCLEAR MEDICINE			
Patient's Details	BHAT PRITAM 44 Years / M		
UHID	ANM1.0001115198	Ward/Bed No.	/
I.P.No/Bill No.		Received On	01-Jul-2026 11:44:40 AM
DRN		Reported On	02-Jul-2026 09:19:28 AM
Referring Doctor	DR. JYOTI BAJPAI		

WHOLE BODY FDG PET-CT

Clinical history: CA left RMT, post 3 cycles of NACT, post CT RT ; PET scan suggestive of residual disease ; post 3 cycles of maintenance immunotherapy (Toripalimab) ; follow up scan

Technique:

Fusion PET-CT was performed 60 minutes after i.v. administration of 4.77 mCi FDG from vertex of skull to mid thigh with IV contrast. Patient was scanned on a dedicated United Imaging Digital TOF PET-CT. Blood glucose was 85 mg/dl prior to scan.

Present scan is compared with the previous PET CT scan dated 21/04/2026.

Report:

Brain:

Cerebral neuroparenchyma reveals normal attenuation, enhancement and grey white matter differentiation. Brain stem and cerebellum appear normal. Ventricles and cisterns appear normal. Calvarium shows no destructive lesion. Choroid plexus calcification noted.

Physiological FDG uptake is noted in Brain.

Note: All brain lesions may not be evident on PET-CT. MRI Brain may be done if clinically warranted.

Head and Neck:

Base of skull, orbits, paranasal sinuses, nasopharynx, oropharynx, hypopharynx and larynx are normal.

Heterogeneously enhancing soft tissue thickening with air pockets and necrotic areas within and peripheral FDG avidity (SUVmax 6.21) noted involving the left retromolar trigone and extending into the left glossotonsillar sulcus with erosion of the adjacent ramus of the mandible (SUVmax 9.25). There is involvement of the adjacent left buccal mucosa buccinator complex on the left side. Superiorly it is found to extend and involve the left infratemporal fossa (SUVmax 8.53) with local erosion of the posterolateral wall of the left maxillary sinus.

Parotids, submandibular glands, thyroid reveal no abnormal increased FDG activity.

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Low-grade FDG avid left level II cervical lymph nodes noted, representative lymph node measures approximately 0.8 x 0.5 cm, SUVmax 1.82.

Low-grade FDG uptake noted involving bilateral tonsils (left > right) and larynx: Likely to represent inflammatory aetiology.

Bilateral carotid arteries and jugular veins appear normal.

Chest:

No significant FDG avid bilateral axillary lymph nodes noted.

FDG avid and non avid right lower paratracheal lymph nodes noted, representative lymph node measuring approximately 0.4 x 0.6 cm, SUVmax 1.94; right hilar lymph node noted (SUVmax 2.13).

FDG avid and non-avid nodular soft tissue density lesions noted involving noted involving upper lobe of right lung, representative nodule measures approximately 0.9 x 0.8 cm, SUVmax 6.34.

FDG avid ground-glass opacities noted involving the lower lobe of right lung, SUVmax 4.53; lower lobe of left lung, SUVmax 6.97.

No significant FDG avid deposits noted involving the chest wall.

No pleural or pericardial effusion. The cardia, pulmonary trunk and aorta appear normal.

Abdopelvis:

Liver is mildly enlarged in size (measures approx. 16.03 cm). No focal lesion with abnormal FDG avidity seen in liver. Portal vein, hepatic vein and IVC are normal. Spleen is normal in size and reveals no abnormal increased FDG uptake (measures approx. 9.7 cm).

No significant FDG avid periportal, peripancreatic, perigastric, celiac axis, superior mesenteric, splenic hilar lymph nodes noted.

Gallbladder, stomach, adrenals, pancreas, both kidneys are normal and show no abnormal FDG uptake, no abnormal increased FDG uptake is noted in the omentum /peritoneum.

Urinary bladder noted. Prostate, seminal vesicles reveal no focal abnormal FDG uptake.

No significant FDG avid retroperitoneal / pelvic /inguinal lymph nodes noted.

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FDG distribution in bowel loops is in a physiologic pattern.

No evidence of ascites. No free fluid noted in the pelvis.

Abdominal aorta appears normal.

Bones:

Local erosion of the ramus of the mandible on the left side and posterolateral wall of the left maxillary sinus noted.

Non-FDG avid sclerotic lesions noted involving heads of bilateral femora: Likely to represent bone island. Diffuse low-grade FDG activity noted involving the mid and lower dorsal and lumbar vertebrae, pelvis (SUVmax 2.74) : Likely to be secondary to reticulo endothelial stimulation.

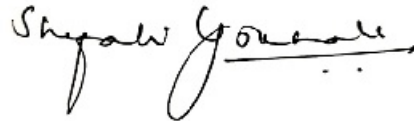
IMPRESSION:

Present scan as compared to the previous PET CT scan dated 21/04/2026

- **Hypermetabolic soft tissue thickening with air pockets noted involving the left retromolar trigone with extension to the left glossotonsillar sulcus and erosion of the adjacent ramus of the left hemimandible, involvement of the adjacent left sided buccal mucosa buccinator complex. Superiorly it is found to extend into the left infratemporal fossa with erosion of the posterolateral wall of the maxilla on the left side. The metabolism of the lesion has increased significantly as compared to the previous scan.--- Residual/recurrent disease.**
- **Hypermetabolic subcentimetre left level II cervical lymph node noted. (previously ametabolic): Could represent lymph node metastases, for follow up evaluation.**
- **Hypermetabolic and ametabolic soft tissue density nodular lesions noted involving upper lobe of right lung as described above. (not seen in the previous scan): Indeterminate aetiology, for follow up evaluation.**
- **Low-grade hypermetabolic right lower paratracheal lymph node noted as described above (previously ametabolic) .: For follow up evaluation.**
- **Hypermetabolic ground-glass opacities noted involving the lower lobes of bilateral lungs: Could represent infective aetiology.**

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- No significant hypermetabolic lesions suggestive of metastasis noted involving the liver.
- No evidence of hypermetabolic distal skeletal metastasis noted.



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---END OF THE REPORT---